

# Request for Proposal

## AI-Powered Ambient Clinical Documentation for Outpatient Care

**Issued by:** Midwest Regional Health

**Document number:** MRH-RFP-2026-014

**Issue date:** February 17, 2026

**Response deadline:** April 7, 2026, 5:00 PM CT

**Primary contact:** Sarah Hollister, Director of Strategic Sourcing

**Email:** [sourcing@midwestregionalhealth.org](mailto:sourcing@midwestregionalhealth.org)

# 1. Introduction and Background

Midwest Regional Health (MRH) is a not-for-profit integrated health system operating ten acute-care hospitals and approximately 240 outpatient clinics across the upper Midwest. Our system employs approximately 3,500 providers, including 1,950 physicians and 1,550 advanced-practice clinicians, who together deliver care to roughly 1.4 million unique patients each year across primary care, medical and surgical specialties, and behavioral health.

MRH is issuing this Request for Proposal to identify a long-term strategic partner to deploy an AI-powered ambient clinical documentation solution across our outpatient enterprise. Provider documentation burden is consistently cited in our annual engagement surveys as the leading driver of burnout, and our internal time-and-motion studies estimate that ambulatory clinicians spend an average of 1.8 hours per workday on after-hours documentation, commonly referred to as 'pajama time.'

Our vision is to deploy a solution that captures the clinician-patient encounter ambiently, generates a structured, specialty-appropriate clinical note ready for clinician review within minutes of the encounter ending, and writes the finalized note back to the patient's electronic health record without requiring the clinician to switch context. We expect a successful deployment to materially reduce documentation time, improve note quality and completeness, and contribute to measurable improvements in provider engagement scores within twelve months of go-live.

## 2. Scope of Work

The selected vendor will provide a software-as-a-service platform that performs ambient capture of clinician-patient encounters in outpatient settings, generates structured clinical documentation, and integrates with MRH's electronic health record systems. The platform will be deployed across all ambulatory specialties and locations within the MRH network on a phased basis.

### 2.1 Volume and footprint

- Approximately 3,500 ambulatory providers across the MRH network
- Approximately 7.2 million outpatient encounters annually
- Primary care and family medicine: approximately 38% of encounters
- Medical and surgical specialties: approximately 47% of encounters (cardiology, oncology, orthopedics, gastroenterology, endocrinology, and twenty-plus additional specialties)
- Behavioral health: approximately 15% of encounters
- Patient populations: predominantly English-speaking, with a meaningful and growing Spanish-speaking population concentrated in the Twin Cities and Milwaukee metros

### 3. Technical Requirements

Vendors must address each of the following technical requirements in their response. Items marked **[Required]** are hard requirements; a response that does not meet a Required item will not advance to commercial evaluation. Items marked **[Strongly Preferred]** will be scored within the technical evaluation category and may affect the overall ranking.

#### 3.1 Electronic Health Record integration

- **[Required]** Bi-directional integration with Epic Hyperdrive — eight of MRH's ten hospital regions and the majority of ambulatory clinics operate on Epic
- **[Strongly Preferred]** Bi-directional integration with Oracle Cerner Millennium — two of MRH's hospital regions and a portion of behavioral health operate on Cerner
- Integration must use modern FHIR R4 APIs where available; legacy HL7v2 interfaces are acceptable for transitional support
- The platform must write finalized clinical notes back to the patient's chart in a structured format compatible with downstream coding, quality reporting, and patient portal workflows

#### 3.2 Ambient capture and note generation

- **[Required]** Ambient audio capture without requiring the clinician to dictate, summarize, or initiate a separate transcription step
- Note generation latency must average under three minutes from encounter conclusion to a clinician-reviewable draft
- Specialty-aware note formatting across primary care, medical and surgical specialties, and behavioral health — vendors will be evaluated on the depth and quality of specialty-specific note structures
- Clinician edits to draft notes must be captured and used, where the vendor's architecture permits, to improve note quality for that clinician over time

#### 3.3 Language support

- **[Required]** Native English support for all clinical content
- **[Required]** Spanish support for ambient capture of Spanish-speaking patients in clinician-patient encounters where the clinician converses with the patient in Spanish or via an interpreter
- Additional languages are not required but may be scored favorably in technical evaluation

#### 3.4 Access, identity, and devices

- **[Required]** Single sign-on via Microsoft Entra ID (formerly Azure AD)
- Clinician access via the EHR Hyperspace or Hyperdrive embedded experience where supported by the EHR vendor
- Mobile access via iOS and Android for ambulatory and home-visit workflows; iOS access is required, Android is strongly preferred

## 4. Compliance and Security Requirements

MRH operates in a regulated environment under HIPAA, HITECH, and applicable state privacy laws. The selected vendor will be designated as a Business Associate and will be required to execute MRH's standard Business Associate Agreement. The following compliance and security requirements are non-negotiable.

### 4.1 Regulatory and certification requirements

- **[Required]** Full HIPAA Privacy Rule and Security Rule compliance, attested in the vendor's response
- **[Required]** Execution of MRH's standard Business Associate Agreement, attached as Exhibit B to this RFP. Vendor-proposed material modifications to the BAA may be grounds for disqualification
- **[Required]** SOC 2 Type II attestation completed within the prior twelve months; vendor must provide a copy of the most recent SOC 2 report under NDA during evaluation
- **[Strongly Preferred]** HITRUST CSF r2 certification, current and in good standing

### 4.2 Data residency and handling

- **[Required]** All Protected Health Information must reside in data centers physically located within the contiguous United States
- **[Required]** All PHI must be encrypted at rest using AES-256 or equivalent industry-standard encryption, and in transit using TLS 1.2 or higher
- **[Required]** Vendor must not use MRH PHI to train, fine-tune, or otherwise improve foundation models, speech models, or any other machine-learning artifact without MRH's explicit, written, opt-in consent for each specific use
- Vendors must clearly disclose the geographic location of all primary, secondary, and disaster-recovery data centers handling MRH PHI

### 4.3 Audit and incident response

- Vendor must provide MRH with access to audit logs sufficient to investigate suspected privacy or security incidents
- Vendor must notify MRH of any confirmed or suspected PHI breach within 24 hours of discovery, irrespective of regulatory minimums

## 5. Commercial Requirements

### 5.1 Pricing structure

Vendors may propose either a per-provider-per-month subscription model or a per-encounter consumption model. Hybrid models are acceptable. All pricing must be presented inclusive of all standard support, hosting, model inference, and EHR integration costs; vendors must separately disclose any one-time or recurring fees that fall outside the standard subscription, including but not limited to implementation, professional services, training, integration maintenance, and major version upgrades.

MRH expects vendors to provide a three-year total cost of ownership projection in the response, based on the volume and footprint estimates in Section 2 of this RFP.

### 5.2 Contract term and renewal

- Initial contract term of three years
- **[Required]** No automatic renewal of greater than one year. Auto-renewal terms longer than one year are non-acceptable and may be grounds for disqualification
- **[Required]** Minimum 90-day written notice of non-renewal, with no penalty for non-renewal
- Year-over-year price escalation, if any, must be disclosed in the response and is expected to be capped at the lesser of 3% or US CPI

### 5.3 Data ownership and exit

- **[Required]** MRH retains full ownership of all clinical notes generated through the Service
- **[Required]** MRH retains full ownership of all underlying audio recordings of clinician-patient encounters captured through the Service
- **[Required]** At any time during the contract and for a minimum of 180 days following contract termination, MRH must be able to export all PHI, audio recordings, and clinical notes generated through the Service in a documented, non-proprietary format

## 6. Evaluation Criteria

Responses that meet all hard requirements will be evaluated by a cross-functional committee using the following weighted scoring framework.

| Category   | Weight | Sub-areas                                                                                                                                  |
|------------|--------|--------------------------------------------------------------------------------------------------------------------------------------------|
| Technical  | 40%    | EHR integration depth · Ambient capture quality · Note quality and specialty coverage · Language support · Mobile and identity integration |
| Commercial | 30%    | Three-year total cost of ownership · Contract flexibility · Implementation cost transparency · Pricing model fit                           |

| Category   | Weight | Sub-areas                                                                                                                 |
|------------|--------|---------------------------------------------------------------------------------------------------------------------------|
| Compliance | 30%    | HIPAA and BAA terms · SOC 2 and HITRUST status · Data residency · Model-training data terms · Audit and incident response |

**Hard requirements (must-pass).** A response that does not meet any of the following will not advance to scored evaluation: executed BAA against MRH's standard template, HIPAA compliance, SOC 2 Type II attestation, US-only PHI residency, MRH opt-in for any model-training use of MRH data, Epic Hyperdrive integration, English and Spanish ambient capture, contract auto-renewal of one year or less.

## 7. Submission Requirements

- Responses must be submitted as a single PDF document, not exceeding 60 pages including all appendices, via the procurement portal at [procurement.midwestregionalhealth.org](https://procurement.midwestregionalhealth.org)
- Responses must be received by 5:00 PM Central Time on April 7, 2026
- Late submissions will not be considered. Questions regarding this RFP may be directed to [sourcing@midwestregionalhealth.org](mailto:sourcing@midwestregionalhealth.org) through March 17, 2026; questions and answers will be shared with all responding vendors

MRH thanks all vendors who choose to respond and looks forward to identifying a partner who can help meaningfully reduce documentation burden for our clinicians.